



GUIDELINE

Clinical Handover

Scope (Staff):	All Staff
Scope (Area):	All CAMHS services

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations.

Read the full statement here:

[CAHS Child Safe Organisation Commitment Statement](#)

This document should be read in conjunction with this [disclaimer](#)

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Aim

All clinical handover within CAMHS must occurred as outlined in:

- [WA Health Clinical Handover policy](#);
- [CAHS Communicating for Safety policy](#); and
- **For PCH-based services:** [Clinical Handover – PCH Multidisciplinary procedure](#)

The aim of this document is to outline clinical handover principles and procedures that are unique to CAMHS and **not** already detailed in the above documents, with specific focus on the application of the [Chief Psychiatrist's Standards for Clinical Care 2015 \(Transfer of Care standard\)](#).

Risk

- Continuity, safety and quality of care for children and families is not maintained during transfer between or within services.
- Children may be at higher risk during transfer of care.

Background

Discharge and care transfer planning ensures that children and their families are provided with the services and materials that best meet their needs when leaving a service, to help them to develop self-management skills, support their autonomy and promote recovery. Inadequate planning for and failure to ensure successful clinical handover has been found to be associated with increased risk of adverse outcomes, including suicide and self-harm, for some consumers.

Definitions

Transfer of care: Transfer of care is a person-centred, recovery focused process used for the timely, safe and effective discharge and clinical handover of care between all service providers. Service providers may include clinical and non-clinical services.

Nominated person: A person who has been nominated by the child to assist the child to ensure that their rights and interests are upheld (MHA ss 273, 263). Nominated persons can be a friend or relative of the child or a guardian (MHA s 273). It is

whoever the child feels should receive information and who can help the child. A child can only make a nomination if they understand the effect of making the nomination (MHA s 273(2)). The person nominated must be an adult over the age of 18 (MHA s 274).

Patient of concern: This term is defined in the [WA Health Clinical Handover policy](#) as “A patient that a clinician is particularly concerned about, as defined by the treating clinician”. In a CAMHS context, this includes any child with a complex mental health presentation and risk management concerns.

Primary or non-acute health care service provider or practitioner: refers to organisations such as headspace, individual private practitioners (private psychologist, paediatrician, or psychiatrist) or a school psychologist (Tier 2 services) or the child’s usual GP. This includes a child who is under the care of the Department of Communities - Child Protection and Family Support, Department of Communities – Disability Services or the Justice Department.

Child: A child refers to an infant, child, adolescent or young person under the age of 18 years.

Family: The person(s) who has parental responsibility for the child. Parental responsibility, in relation to a child, means all the duties, powers, responsibilities and authority which, by law, parents have in relation to children (MHA s 4).

Key principles

- Discharge is a key part of clinical care, just like assessment and treatment, and it should always support recovery. Planning and documenting of discharge or transfer should begin as early as possible in the care episode. See CAMHS [Clinical Assessment](#) policy.
- Wherever possible, teams should aim to reduce the number of different staff each child and family sees to reduce the need for re-telling their story to multiple staff and services. In the context of appropriate assessment and risk assessment, duplicate triaging must be avoided.
- CAMHS staff will make every attempt to work collaboratively with other agencies involved in the clinical handover, in a person-centred approach.
- All information must be communicated in a manner that is clear, accurate, timely, directive and suitable for the needs of all of those involved in the clinical handover. This includes involvement from the child and their family, parents or guardians, and—where applicable—their nominated person. It also includes coordination with other services involved in the individual’s care (such as general practitioners and other clinical providers) and culturally appropriate engagement with Aboriginal children, young people and families. See the Chief Psychiatrist’s [Aboriginal Practice standard](#) and “Appendix 1 CAMHS Clinical Assessment in focus – Aboriginal children” in the [CAMHS Clinical Assessment policy](#).

- Health literacy and plain language principles should apply to ensure transparent communication and realistic expectations and understanding of the transfer process. If relevant professional interpreters or cultural support should be provided - Refer [WA Health Language Services Policy](#).
- CAMHS must work collaboratively and in partnership with Aboriginal children with mental illness and their family to improve the safety and quality of care.
- Where practical and appropriate, the child and their family should be invited to attend all care planning meetings, including clinical handover meetings. The child or family have the right to request a Peer Worker to attend these meetings, where available.
- Wherever possible, clinical handovers should occur in person between members of the relevant care teams and take place in front of the child and family. This approach supports their engagement in care, reinforces their understanding of the diagnosis and care plan, and provides an opportunity for them to clarify or correct information being shared.
- Clinical handovers conducted in the presence of the child and family should address the following points:
 - The reason for the clinical handover;
 - The name, profession and responsibilities of the new case manager/primary clinician; and
 - The expected duration of clinical handover period, if applicable.
- If the child and family are not able to be involved, they should be informed of when the clinical handover is to occur and has occurred, including the details listed above.
- Follow-up arrangements must be planned and negotiated with the child and their family to ensure choice and collaboration.
- The child and family must partner in developing the transfer information.
- Staff must ensure that the child and the family are provided with YES and CES questionnaires.

Discharge planning

- Discharge planning occurs on entry to a service, as part of the completion of a Treatment Support and Discharge Plan (TSDP) – see [CAMHS Clinical Assessment policy](#).
- When there is a history of poor engagement or non-attendance, clinicians must intentionally review and integrate the longitudinal risk history. Understanding the longitudinal risk assessment provides a more accurate picture of the child's current vulnerability and informs a TSDP that is proactive, assertive, and tailored—ultimately ensuring that the child remains safe and visible to services.

Involvement of Receiving Service

- When a child is being discharged from CAMHS, staff must ensure that, as far as possible, the next agency responsible for supporting or providing care to the child (including supported accommodation providers) is involved in the development of TSDP.
 - **Discharge to an external agency:** When transferring care to a team outside of CAMHS, the receiving team must be invited to participate in the discharge planning process.
 - **Discharge to another CAMHS team:** When transferring to another CAMHS service, the receiving team must be included in discharge planning, including the development and agreement of the TSDP. Participation may occur face-to-face, via videoconferencing, or by telephone.
 - **Children who are CAMHS inpatients and open to a community CAMHS team:** Where a child is an inpatient but is also engaged with a community-based CAMHS team, the community CAMHS case manager (or proxy if unavailable) must participate in all clinical reviews, discharge discussions, and transfer planning to ensure continuity of care. The Community CAMHS Consultant Psychiatrist must participate in the last discharge review. Where they are unable to attend the last discharge review, the CAMHS Acute Inpatient Unit Consultant Psychiatrist must provide a clinical handover of the child's care to the Community CAMHS Consultant Psychiatrist.

Evidence of Engagement

- Staff must demonstrate clear and documented attempts to involve the receiving team in discharge planning. Where the receiving team is not involved despite attempts, staff must follow the [Escalation process](#).

Consent and Information Sharing

- All exchange of information with receiving teams must be undertaken with the consent of the child and their family, in accordance with the [CAMHS Consent Policy](#).

MDT review prior to clinical handover

- MDT clinical reviews must be undertaken as described in the [CAMHS MDT Clinical review policy](#), including at discharge.
- There must be documented evidence in the child's medical record that the Consultant Psychiatrist agrees with the decision to discharge the child, as previously planned with the child and family. Alternatively, evidence must be documented in the medical record as to why the decision was made that may have been different from the treatment plan for discharge, e.g. when the patient elects to be discharged against medical advice.
- This does not apply to shift-to-shift handover and within team transfer.

- De-activation (or to close a case in Community CAMHS) must be completed within 5 working days after the decision has been made by the MDT at review.

Timing of planned clinical handover

- Referral and provision of clinical handover information should occur **as soon as practicable and prior to transfer from the referring service.**
- It is the duty of any service (both the referring and receiving), whichever is first aware of the movement of a child known to receive mental health care:
 - To contact the other service promptly to establish the relevant facts; and
 - To provide/request the necessary transfer summary and documentation.
- In cases where a child has accessed care within CAMHS without the knowledge of another involved CAMHS team, any CAMHS staff that are made aware of this must contact the other team where the child is active and let them know of the current circumstance. For example, a child who is an active Community CAMHS client may have presented to an emergency department and then been admitted to CAMHS Acute Inpatient Service.
- Clinical handover to an inpatient unit can occur as a planned or crisis admission. For planned admissions, transfer planning must start as early as possible, (see CAMHS [Clinical Assessment](#) policy) and involve the receiving unit where possible.

Information to handover

Minimum requirements

- For clinical handover to a primary or non-acute healthcare providers (see [definition](#)):
 - Written handover is required; and
 - For [patients of concern](#), verbal handover is required. For all other patients, verbal handover is strongly recommended but not mandatory.
- For all other clinical handovers:
 - Written handover is required; and
 - Verbal handover is required.

Verbal handover

- A verbal handover must be provided in ISOBAR format (see suggested content in [Appendix 1](#)). This must be recorded by the receiving team in the patient's medical record. The CAMHS Clinical Handover Document and Record of Transfer of Care form (MR510.05) may be used to document the verbal handover if desired.

Written handover

- One of the following must be completed and forwarded to the receiving service:
 - Acute Inpatient Service:
 - NaCs
 - All other CAMHS services:
 - CTS. Where the CTS is not considered suitable, a discharge summary in ISOBAR format is acceptable (see suggested content in [Appendix 1](#)).
- In addition, **at minimum**, the following documents must be up-to-date in webPSOLIS prior to the handover. If the receiving service cannot access webPSOLIS, these documents must be forwarded to the receiving service alongside the main handover document.
 - CRAMP
 - Physical Health Assessment
 - TSDP
 - Crisis plan
- The following documents may also be required by the receiving service depending on the nature of the handover:
 - Referral form
 - Last medication review / other medication information
 - Investigation / test results
 - Patient flow checklist
 - Relevant reports
 - SSCD Initial Assessment
 - Discharge MDT review
- Once handover has occurred this must be documented in the child's medical record and should include:
 - Copies of all documents handed over to the receiving service;
 - Time and date of clinical handover; and
 - Contact details of at least one of each of the providing and receiving clinicians.
- When transitioning out of the CAMHS Acute Inpatient Unit, or out of any ambulatory services, the discharge summary (NaCS / CTS) and any accompanying documentation should also be provided to:

- the child's usual GP;
- the child and family/nominated person; and
- the referrer.

This is not required when transitioning from an ambulatory service to the CAMHS Acute Inpatient Service.

- The discharge summary (NaCS/CTS) is to be forwarded to the receiving clinician on the day of discharge. A copy of the discharge summary must be sent with the patient/carer, within 24 hours following discharge. Where practically possible, every attempt should be made to provide the family with a hard copy of the completed discharge summary, which has been signed by the relevant clinician/s, in their final appointment. If a patient/family is not present at the point of discharge (e.g. they do not attend their final appointment, or they disengage from treatment) then the summary should be prepared and posted to the family – see [CAHS Confidentiality, Disclosure and Transmission of Health Information policy](#).
- Different service providers in the continuum of care may require different aspects of information. Transfer of information must recognise other relevant privacy legislation and policy.
- Handover to non-clinical services must include transfer information that reflects the needs of the child within that receiving service.

Responsibilities of care

- **Overall implementation of this policy is the responsibility of the CAMHS Service Manager and Head of Service.**
- The referring CAMHS service clinician retains clinical responsibility for the consumer until clinical handover to the receiving service or practitioner has taken place, or the consumer chooses an alternative service.
- Any unresolved disagreement regarding clinical responsibility or the appropriateness of a transfer must be addressed and resolved promptly. Resolution processes must not delay or compromise the child's safety or their timely access to care.
- The staff member who is responsible for conducting the clinical handover is as follows:
 - **Acute Inpatient Ward** – the child's Care Coordinator / primary clinician, or the CAMHS Crisis Connect Clinician.
 - **EDS** – The child's primary clinician / case manager.
 - **CAHDS, MST, Pathways, GDS, PPIRT, Touchstone** – the child's Care Coordinator / Primary clinician.

- **Community CAMHS** – the child's Case Manager or the Choice Coordinator.
 - Where the child is being referred to Community CAMHS as an 'open case', the handover may be received by the 1. Case Manager, 2. Choice Coordinator, 3. Covering Choice Coordinator, or 4. Senior Clinician.
 - Where the child is being referred to Community CAMHS as a new referral, the handover information may be received by 1. The Choice Coordinator, 2. Covering Choice Coordinator, 3. Senior clinician.

Escalation

- **The relevant CAMHS Consultant Clinical Psychiatrist must be made aware of all clinical escalations.**
- Any issues with clinical handover must be escalated to:
 - **EDS, GDS, CAHDS, Pathways, PPIRT, Touchstone, EDS** – Service Manager or Head of Service
 - **Community / ACRT** – Service Manager or Head of Service, Clinical Nurse Consultant
 - **Acute Inpatient Units** – Registrar / Psychiatrist, Head of Service, Nurse Coordinator Mental Health, Clinical Nurse ,CAMHS Crisis Connect Consultant
- Instruction regarding escalation of care for patients who are physically or mentally deteriorating is detailed in the below policies:
 - CAHS [Recognising and Responding to Acute Deterioration](#) policy;
 - CAMHS [Risk Assessment and Management policy](#); [Clinical Assessment policy](#); [Inpatient Unit Progressive Risk Assessment work instruction](#), [CAMHS Physical Health Assessment and Monitoring policy](#); [Clinical Escalation procedure](#), [Transportation of Mental Health Consumers procedure](#) and [Crisis/Incident Management in Touchstone](#) work instruction.
- Ambulance transfers should be considered in acute situations for services that are unable to provide emergency physical input.
- Where a physical health issue is related to a mental health issue (i.e. self-harm) CAMHS Crisis Connect must also be informed, particularly where the child is under 16 years of age.
- If the child has no immediate mental health concerns and is only being treated for a physical health issue unrelated to a mental health issue, CAMHS Crisis Connect do not need to be informed.

- The child's family must be informed of any emergency situation.

7 Day Community CAMHS follow-up after discharge from the inpatient ward

- The 7-day follow-up after discharge from a CAMHS Acute Inpatient Service must be completed by the Community CAMHS team, in accordance with the National Healthcare Agreement and the WA Chief Psychiatrist Standards of Clinical Care.
- The child must be seen by the community service without delay and no later than seven days after transfer from the inpatient service. This may happen sooner when there are specific risk issues that require earlier follow up. In these circumstances:
 - The risk issues must be clearly identified and communicated to the receiving team and the receiving team's capacity to follow up must also be negotiated.
 - A first, follow-up, appointment date is to be specified in the discharge plan. This must be within 7 days of discharge from the CAMHS Acute Inpatient Service. If not set for within seven days of discharge from the inpatient service, reasons must be clearly documented in the inpatient notes.
 - Follow up arrangements must be planned and negotiated with the child and family to ensure safe transfer of care.
- Where a child or family does not keep the planned follow-up arrangements there must be active consideration and management of risk. Children or family who do not keep the planned follow-up arrangements must be assertively followed-up by the receiving CAMHS team.

Re-entry to the service

- Re-referrals must be considered for re-entry into the service based on clinical need, including an assessment of the child's current psychosocial context and clinical presentation against the service's admission criteria, outlined in its Model of Care or Entry Protocol.
- If a child re-enters a service, they must be allocated to their previous clinician(s), where this is deemed possible and appropriate, to provide continuity of care.

Referral to CAHDS, EDS, MST, Pathways, GDS, PPIRT, Touchstone

- See entry protocols, Models of Care, intranet and internet sites for referral information for specific teams.
- Please note that referrals to these teams come from sources both internal and external to CAMHS, and it is therefore recommended to refer as early as

possible as a part of the discharge planning process (see CAMHS [Clinical Assessment](#) policy).

- Clinical responsibility stays with the initial CAMHS team until referral has been accepted and care is handed over to the receiving team.
 - This does not apply to CAHDS. The CAHDS criteria stipulates that the referrer remains engaged and active as the primary care provider. Where the child is being referred by a Community CAMHS team and there is active engagement with a paediatrician (private or Child Development Service), the Community CAMHS team does not need to remain active in the child's care. Where the child is being referred by a Community CAMHS team and is **not** actively engaged with a paediatrician, the Community CAMHS team must remain active in the child's care.
- Alternatively, if treatment in the initial CAMHS team is complete, care may be handed back to the original referrer or another appropriate community or Tier 2 service (GP or school for example), while awaiting response or start of treatment from the receiving CAMHS team.
 - This does not apply to CAHDS. CAHDS does not accept referrals or work with primary or Tier 2 services.
- Referral from CAMHS Acute Inpatient Service
 - Where acceptance of referral has not been received prior to discharge from the inpatient unit, clinical care must be handed over to the original referrer (for example, community CAMHS, GP, paediatrician, ACRT) while waiting for the outcome of the referral.
 - Once referral has been accepted by the receiving CAMHS team, clinical handover with discharge planning must occur.
- For Touchstone, MST and Pathways (mostly) agreement to discharge does not apply since the period of treatment is predetermined.

Clinical handover due to long leave/change of duties within one CAMHS team

- This applies to situations such as long leave (more than 3 workdays) or change of duties.
- This applies to children at high risk and those who would normally be seen by the case manager/primary clinician within the time period and any others where clinically relevant.
- Additional discharge planning or MDT review does not apply for this clinical handover situation.
- As soon as the case manager/primary clinician anticipates the need to transfer clinical responsibility/accountability, they must:

- provide the receiving clinician with verbal and written clinical handover in ISOBAR format; and
 - ensure that any written supporting clinical documents (TSDP, physical health assessment, RAMP, MDT review outcomes) are up-to-date in webPSOLIS.
- Minimally, in unforeseen circumstances, where verbal clinical handover cannot occur, written clinical handover on CAMHS Clinical Handover Document and Record of Transfer of Care form (MR510.05) must be done
- The receiving clinician must record the verbal ISOBAR clinical handover on the CAMHS Clinical Handover Document and Record of Transfer of Care form (MR510.05) and file it in the medical record.
- For temporary transfers of care (clinicians on leave or secondment), this clinical handover is also to be provided on the return of the case manager/primary clinician, as soon as practicable.
- Any issues must be escalated to the Service Manager or Head of Service.
- Where clinical handover cannot occur due to unexpected leave (e.g. due to significant illness or injury), the Service Manager is responsible for allocating an interim case manager and ensuring continuity of care for the child.

Clinical handover at shift-to-shift or after MDT review within CAMHS Acute Inpatient Unit

- This clinical handover is without additional discharge planning.
- The nursing shift-to-shift at 7am, 1pm, 9pm clinical handover must:
 - Comply with the [Clinical Handover – PCH Multidisciplinary](#) procedure;
 - Cover all patients on the ward; and
 - Be overseen by the most senior nurse available.
- Shifts for staff involved in clinical handover should have adequate cross over time for a thorough clinical handover to ensure continuity of care.
- The clinician leading or initiating the clinical handover must endeavour to limit non-critical interruptions using controls appropriate to the local environment.
- The clinician leading or initiating the clinical handover must provide:
 - Pre-prepared documentation (iCM handover sheet); and
 - Any other written supporting clinical documents.
- The handover must occur at the bedside, immediately after the professional clinical handover at time of shift change.

Additional requirements for children under the care of the CEO, Department of Communities

- Discharge and transfer planning for all children who are under the care of the CEO Department of Communities must include active engagement with the child's Department of Communities Case Manager, from assessment throughout treatment and discharge.
- It is essential to actively involve the receiving service in transfer of care for children who are under the care of the CEO Department of Communities. Where the receiving service is not involved despite attempts, staff must escalate as per [Escalation](#) process.

Additional requirements for transient and homeless children

- Discharge and transfer planning for all homeless children must include consultation with services with expertise in homelessness including accommodation services.
- It is essential to actively involve the receiving service in transfer of care for transient and homeless children. Where the receiving service is not involved despite attempts, staff must escalate as per [Escalation](#) process.

Additional requirements for transition to youth and adult mental health services

- The following information should be read in conjunction with the CAHS [Transition of Adolescents Young Adults to Adult Health Services](#) Policy.
- Preparation for a smooth transition must begin at the earliest opportunity. For Community, this must be at least 3 months before the child is due to leave the CAMHS team and must be discussed at each scheduled clinical review. CAMHS is responsible for initiating this through discussion with the child and their family/nominated person, and by approaching youth/adult mental health services with a written referral summarising the case and reasons for transfer of care.
- Where it is agreed that the child has needs that are likely to be met in a specialist service within youth or adult mental health, the CAMHS clinician initiating the transfer will approach the specialist service. Where the child is deemed not to meet the entry criteria for the youth/adult mental health service, CAMHS must determine the reasons for this decision and seek advice about other appropriate services to which a child may be referred. CAMHS will then work with the child and family to ensure the child will be supported by appropriate services after discharge from CAMHS.
- Following acceptance of the referral by youth/adult mental health services, CAMHS must make themselves available to attend any initial planning meetings.
- Preparation for the transfer to the youth/adult mental health service must involve discussion with the child and their family/nominated person in

understanding the need for the child to become responsible for attendance at services and concordance with treatments offered. These discussions must be documented in the medical record.

- CAMHS may become aware of a waiting time before the child receives support from the youth/adult service. If this occurs CAMHS will work with the child and their family and/or nominated person, to ensure the child is supported by appropriate services in the interim. [Escalation](#) should be considered if the receiving service is not responding in a timely manner to provide services in relation to the level of clinical risk.
- At minimum, at an initial appointment with the youth/adult mental health service a CAMHS worker known to the child must be present to provide support and ensure attendance.
- A period of shared care may be beneficial to the child, see the [Shared Care in CAMHS](#) guideline.
- Links for contact details to youth public health services:
 - [FSH Youth Mental Health Services Intranet Page](#)
 - [NMHS Youth Mental Health Programs Intranet Page](#)
 - [MHC Getting Help Internet Page](#)

Additional requirements for children under the MHA

- This relates to a child under an involuntary treatment order or community treatment order under the MHA, or a child who is a supervised person required under the [Criminal Law \(Mental Impairment\) Act 2023](#) (WA) to be detained at Ward 5A.
- The treatment, support and discharge plan must outline treatment, care and support (MHA s 186):
 - To be provided to the child while under the MHA or admitted by the hospital; and
 - To be offered to the child once no longer under the MHA nor admitted by the hospital.
- The child's treating psychiatrist must ensure (MHA s 187):
 - That the plan is prepared as soon as practicable, is reviewed regularly and is revised as necessary, while having regard to guidelines published under the MHA s 547. This includes the [Chief Psychiatrist's Standards of Clinical Care](#), which underpins this guideline.
 - The plan is filed, a copy of the plan is given to the child, their family and, where the child does not have capacity, to the person authorised by law (MHA s 188(1)(b)).
- The child has a right to be involved in the preparation of the plan whether or not they have the capacity to consent and whether or not the plan can be

implemented. The patient's opinion and preferences must always be sought and given due consideration (MHA s 188) and be recorded.

- If the child has a nominated person:
 - A copy of the discharge plan must be given to the nominated person (unless the nominated person is not entitled to receive it for the reason specified in the MHA s 269(1)).
 - The nominated person must be involved in the preparation and review of the discharge plan (unless the nominated person is not entitled to be involved under the MHA s 269).
- The psychiatrist and treating team should review the CTO at discharge from an inpatient to an outpatient area and decide whether the CTO should remain or be revoked.
- The treating psychiatrist of an involuntary inpatient at an authorised hospital may make a transfer order authorising the involuntary inpatient's transfer from the authorised hospital to another authorised hospital specified in the order (MHA s 91, [Form 4C - Transfer Order](#) must be used).
- Regarding the release of a child in custody under another law refer to:
 - The MHA s 96; and
 - The CAHS [Custodial Persons and Young Persons in Custody](#) policy.

Training

- In addition to mandatory CAHS training, CAMHS clinical staff must complete an e-Learning training package relating to clinical handover. This must be completed on commencement with CAMHS, as part of the staff member's overall CAMHS orientation.

Compliance Monitoring

- The WA Chief Psychiatrist mandates a 7-day community follow-up for all inpatients after discharge as per the transfer of care standard. This is tracked by the Department of Health via Health Service Performance Reports (HSPR).
- Discharge and transfer processes must be regularly monitored to identify issues in the implementation of this policy, **including whether transfer summaries have been provided to receiving services.**
 - Hospital discharge summaries are tracked via HSPR reports;
 - The CAMHS Documentation Audit assesses compliance against both the CTS and TSDP, though the audit does not currently capture whether the CTS has been provided to the receiving service. This is being built into the CAMHS audit schedule.
- Auditing Requirements - Compliance with this policy document must be audited, using an approved audit tool.

References and related external legislation, policies, and guidelines
A national framework for recovery oriented mental health services: guide for practitioners and providers
Australian Institute of Health and Welfare. National Healthcare Agreement: PI 25–Rate of community follow up within first seven days of discharge from a psychiatric admission, 2017 [Internet]. Australian Government, Australian Institute of Health and Welfare; 2017 [updated 2017 Nov 21; cited 2017 Dec 12]. Available from : http://meteor.aihw.gov.au/content/index.phtml/itemId/630053
Chief Psychiatrist's Standards for Clinical Care 2015
Department of Health Community Mental Health Status Assessments: Role of Mental Health Clinicians Policy (MP 0099/18)
Mental Health Act 2014 (WA)
National Safety and Quality Health Service Standards Second Edition
Stokes, B. Review of the admission or referral to and the discharge and transfer practices of public mental health services in Western Australia. 2012 July. https://www.mhc.wa.gov.au/media/1288/mental-health-review-report-by-professor-bryant-stokes-am-1.pdf
WA Health Clinical Handover Policy
WA Health Language Services Policy.
WA Health Recognising and Responding to Acute Deterioration Policy
Wasserman D, Rihmer Z, Rujescu D, Sarchiapone M, Sokolowski M, Titelman D, et al. The European Psychiatric Association (EPA) guidance on suicide treatment and prevention . J Europ Psych. [Internet]. 2012.
Related CAHS internal policies, procedures and guidelines (
CAHS Communicating for Safety Policy
CAHS Confidentiality, Disclosure and Transmission of Health Information policy.
CAHS Clinical Incident Management Policy
CAHS Custodial Persons and Young Persons in Custody Policy
CAHS Language Service Policy.
CAHS Patient/Client Identification Protocol

CAHS Recognising and Responding to Acute Deterioration
CAHS Transition of Adolescents Young Adults to Adult Health Services policy
PCH Clinical Handover – PCH Multidisciplinary Procedure
CAMHS Consent Policy
CAMHS Clinical Assessment Policy
CAMHS Clinical Escalation procedure
CAMHS Inpatient Unit Progressive Risk Monitoring work instruction
CAMHS Physical Health Assessment and Monitoring policy
CAMHS Risk Assessment and Management Policy
CAMHS Multidisciplinary Team Clinical Reviews Policy
CAMHS Shared Care Guideline
CAMHS Transportation of Mental Health Consumers procedure
Touchstone Crisis Management work instruction

Useful resources (including related forms)
CAMHS Clinical Handover Document and Record of Transfer of Care form - MR510.05
CAMHS Consent for Exchange of Information Form with External Agencies - MR510.07
NaCS - Notifications and Clinical Summaries web page
SSCD forms
MHA forms
CAHS Aboriginal and Torres Strait Islander Patient Care and Cultural Learning Guidelines
Adams Y, Drew N, Walker R. Principles of Practice in Mental Health Assessment with Aboriginal Australians. In: Dudgeon P, Milroy H, Walker R, editors. Working Together: Aboriginal and Torres Strait Islander Mental Health and Wellbeing Principles and Practice [Internet]. 2nd ed. Perth: Telethon Kids Institute

This document can be made available in alternative formats on request.

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Standards Applicable:	NSQHS Standards:  Child Safe Principles: 1, 4, 5, 9,		
Printed or personally saved electronic copies of this document are considered uncontrolled			
Healthy kids, healthy communities Compassion Excellence Collaboration Accountability Equity Respect			

Appendix 1: Information to handover (in ISOBAR structure)

ISOBAR	Information to handover (guided by Chief Psychiatrists standards)
I: IDENTIFY - Introduce yourself and your patients	• Contact details for consumer, carer, and guardian where relevant
S: SITUATION - Describe the reason for handing over	• Urgency of the review • Any therapies used and ceased, including reasons for this, adverse effects and any significant clinical incidents. • Medications used and ceased, including: ○ Name of drug/s, dose, frequency, route and duration of use; ○ Reasons for using and / or ceasing medication/s; ○ Any physical health monitoring that has occurred in relation to a medication or change in medication (e.g. CAMHS Antipsychotic Physical Health Monitoring Form); ○ Any adverse effects or significant clinical incidents relating to medication; and ○ Any medicine information provided to consumer. ○ <u>Any recent medication changes must be highlighted on the handover document.</u>
O: OBSERVATIONS - Include vital signs and assessments	• Risk assessment and management. • Physical healthcare assessment and management (e.g. known co-morbid physical health needs) • Mental state examination changes from admission to discharge
B: BACKGROUND - Pertinent patient information	• A case formulation, including a summary of those factors which are essential for understanding the patient as an individual and within their social and cultural environment • Standardised clinical diagnoses • Known infectious status • Any critical information relevant at the time of handover

	• Current allergy/adverse drug reaction status • Relevant diagnostic test results and test results pending
A: AGREE A PLAN - Given the situation, what needs to happen	• Known signs and symptoms which indicate potential mental health deterioration (relapse signature) • Contingency strategies/crisis plans • Post discharge follow-up arrangements (time/date of appointments, name and contact details or agency and/or clinician/case worker) • Information around returning to the current service if needed;
R: READBACK - Confirm shared understanding	